EQUITY RESEARCH

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September 9, 2026

Pharvaris N.V.

Best CHAPTER Yet: Pivotal Data Cements Deucrictibant as a Compelling Oral HAE Prophylaxis

Our view: CHAPTER-3 data looks to be a homerun with the ph.III recapitulating and even improving upon prior ph.II data, combined with a relatively clean safety profile; we had a chance to catch up with management following the more detailed data presentation and get KOL feedback. Given the compelling efficacy data that rivals injectable-class reductions, we are increasing our WW peak sales estimates to ~€720M from our prior €670M, reflecting XR's potential to command greater market share. We would continue to be buyers and see upside from here; raise PT to $62 from $52 on higher confidence in out-year estimates and success of XR.

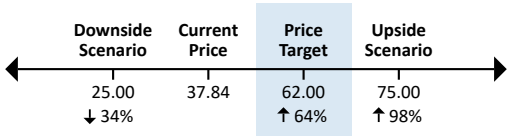
Key points:

Please find our note with takeaways from our recent call with an HAE KOL here; please contact your RBC sales representative for a replay of the event.

Efficacy looked compelling, and we think should be competitive in HAE. Baseline demographics were well-balanced across arms, with baseline HAE attack rate on the higher end relative to historical HAE prophylaxis studies - with mean attack rates of ~3.47/month in the placebo arm and ~3.76/month in the drug arm vs. ~3-4 - which we also note was a more severe population than that enrolled in CHAPTER-1 (ph.II), which recall had baseline attack rates of ~2 attacks/month. Despite the inclusion of more severe patients, XR demonstrated 83% placebo-adjusted attack rate reduction (p<0.0001) across all HAE types with this improving to 87% among just Type 1/2 patients - consistent with or exceeding the company's stated 80-85% target range and broadly in line with CHAPTER-1 ph.II data (85%). Detailed data also demonstrated that onset of protection was rapid, evident within the first week of treatment initiation and sustained throughout the full 24-week study period - which our KOL suggests could help reassure new to prophy patients, where XR could be a front-line option. Responder analysis was also equally robust with the proportions of patients achieving ≥50%, ≥70%, ≥90% and 100% reduction in attacks at the higher end of what has been demonstrated with even the best injectable prophy drugs (Exhibit 1). The company plans to share additional data at medical conferences and is preparing a publication to share additional details.

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Outperform
Speculative Risk
NASDAQ: PHVS; USD 37.84
Price Target USD 62.00 ↑ 52.00
Scenario Analysis*



*Implied Total Returns

Key Statistics

Shares O/S (MM):	68.1	Market Cap (MM):	2,576
Dividend:	0.00	Yield:	0.0%
		Avg. Daily Volume:	567,228

RBC Estimates

FY Dec	2025A	2026E	2027E	2028E
Revenue	0.0	0.0	38.2	190.0
EPS, Ops Diluted	(2.97)	(2.85)	(2.58)	(1.21)
P/E	NM	NM	NM	NM
Revenue	**Q1**	**Q2**	**Q3**	**Q4**
2025	0.0A	0.0A	0.0A	0.0A
2026	0.0A	0.0A	0.0E	0.0E
EPS, Ops Diluted				
2025	(0.85)A	(0.83)A	(0.60)A	(0.72)A
2026	(0.60)A	(0.70)A	(0.73)E	(0.81)E
Prev.			(0.72)E	

All values in USD unless otherwise noted.
Priced as of prior trading day's market close, EST (unless otherwise noted).

Disseminated: Sep 9, 2026 05:00EDT; Produced: Sep 9, 2026 01:01EDT
For Required Conflicts Disclosures, see page 9

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Exhibit 1 - Comparison of Efficacy Among Drugs for HAE Prophylaxis

	Cinryze	Haegarda	Takhzyro	Andembry	Dawnzera	Orladeyo	Deucrictibant XR (40mg)
Administration	IV	SubC	SubC	SubC	SubC	Oral	Oral
Mean monthly attack rate			3.2-4.0	2.5-3.1	2.9-3.6	2.9-3.0	**3.5-3.8**
Mean monthly attack rate reduction vs. placebo		84%	73-87%	89%	55-81%	0.44	**83-87%**
Mean attack rate reduction vs. baseline/run-in	71-85%	~82%	85-93%	91%	68-88%	45-58%	**91%**
Mean reduction in the use of ODT		89%	74-87%	88%	67-92%	0.54	**93% (ph.II)**
≥50% attack reduction		90%	1	95%	83-93%	0.58	**98%**
≥70% attack reduction		83%	76-89%	92%	65-92%	0.5	**96%**
≥90% attack reduction		58%	55-67%	74%	48-62%	0.23	**66%**
% of pts attack free/placebo		40%/0%	31-44%/2%	62%/0%	35-53%/9%	10%/3%	**46%/3%**

Source: Company reports, RBC Capital Markets estimates

Safety generally looked clean, though liver signals will be worth watching as data matures in OLE. Deucrictibant XR was generally well tolerated with the most common AEs upper respiratory tract infection, nasopharyngitis, and headache though these were generally mild-to-moderate and balanced between the active and placebo groups. There was also a slight imbalance in gd.3 events (5.5% vs. 0% in placebo) - including a tooth infection and two gd.3 liver enzyme elevations above 5x ULN; while the liver enzyme elevations are somewhat concerning, we note that these cases were both asymptomatic and potentially confounded by long term androgen use/fatty liver though one of these patients discontinued (discontinuations were balanced with placebo). We would note that Orladeyo also had one discontinuation due to liver enzyme elevations (vs. 0 on placebo) although this is not something indicated on the label. Still, our KOL feedback suggests that these are AEs worth monitoring, and if they persist in the OLE trial, would likely necessitate liver monitoring; we would look for additional context at upcoming medical congress presentations/publications. There was one gd.4 TEAE in the drug arm of laryngeal dyskinesia, though we do not think this was related to drug as the patient had a history of laryngeal attacks and was still otherwise a good responder.

Given the compelling efficacy data, and our KOL feedback, we see upside to our prior estimates. Our KOL believes that deucrictibant XR is likely to be highly competitive, taking market share from existing prophy drugs and expects XR could be the preferred front-line option for prophylaxis-naive patients. Our KOL also expects all of his current Orladeyo patients (4/17) to switch within the first year of XR being available (acknowledging this may not be the view taken by all docs), noting that even patients who appear nearly attack-free on Orladeyo may have simply modified their lives significantly to reduce the risk of breakthrough attacks and so could have quality of life improvements on XR. For those on injectables, he noted the HAE space has a culture of switching given how rapidly the treatment landscape has evolved, and since patients have already switched multiple times, he sees entrenchment as less of a major barrier. We now

see WW peak sales of ~€720M vs. our prior estimate of €670M driven by greater market share penetration potential in both prophylaxis-naive patients and those currently on oral/injectable alternatives.

We continue to see an attractive risk/reward into upcoming catalysts. Following additional mature data from the CHAPTER-4 OLE, the company plans to submit an NDA for deucrictibant XR (prophylaxis) in 1H27, with EMA submission to follow shortly after. For acquired angioedema (AAE-C1INH), topline data from Part 1 of the CREAATE ph.III study are anticipated in 1Q27, where today's data suggests to us a higher PoS (we now see 80% from 60%); success here would represent incremental upside optionality in a less penetrated and less competitive indication (we est. ~€150M combined IR+XR peak sales oppty). Lastly, we see a high likelihood of approval (90% PoS) for IR supported by positive RAPIDe-3 pivotal data (PDUFA April 23, 2027), where we see ~€300M WW peak sales oppty; this represents a potential near-term commercial inflection point, and we think could help to build a patient base that can serve as early adopters once XR is approved. On the on-demand side, our KOL noted that Ekterly now accounts for ~20% of his on-demand use with use still rising, though he believes the deucrictibant IR data was superior, and so would likely be his preferred on-demand option going forward once approved (assuming pricing and insurance coverage are similar to Ekterly).

Exhibit 2 - PHVS Income Statement

PHVS Income Statement *(€ in thousands except price)*	2025A	1Q26A	2Q26A	3Q26E	4Q26E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E	2035E
deucrictibant IR Total Sales						€ 0	€ 35,239	€ 99,600	€ 165,742	€ 220,271	€ 261,118	€ 297,392	€ 322,915	€ 340,830	€ 353,682
deucrictibant IR US sales						-	35,239	86,500	136,183	175,487	204,388	231,925	249,054	261,692	270,121
deucrictibant IR EU sales						-	-	13,100	29,559	44,784	56,730	65,468	73,861	79,139	83,560
deucrictibant XR Total Sales							€ 0	€ 87,387	€ 200,958	€ 307,573	€ 404,599	€ 498,487	€ 584,235	€ 657,135	€ 717,530
deucrictibant XR U.S. sales							-	87,387	192,612	290,110	379,168	465,993	545,100	612,208	667,750
deucrictibant XR EU sales							-	-	5,665	12,886	19,255	24,921	30,257	34,972	38,863
deucrictibant XR Int'l sales							-	-	2,240	3,079	3,726	4,380	5,040	5,707	6,381
deucrictibant XR Japan royalties							-	-	440	1,497	2,451	3,193	3,839	4,248	4,537
Collaborative and other R&D							3,000	3,000	13,000	3,000	23,000	3,000	3,000	3,000	3,000
Total revenues, net	**-**	**-**	**-**	**-**	**-**	**-**	**38,239**	**189,987**	**379,700**	**530,844**	**688,718**	**798,879**	**910,151**	**1,000,965**	**1,074,212**
Costs of goods						0	529	2,805	5,500	7,918	9,986	11,938	13,607	14,969	16,068
Research and development (incl SBC)	124,478	30,161	35,000	35,700	33,915	134,777	114,560	108,832	111,009	113,229	115,494	117,803	120,159	122,563	125,014
Selling, general and administrative (incl SBC, roya	45,345	14,111	15,788	20,525	28,734	79,158	111,878	167,235	223,566	274,430	309,031	333,306	355,542	370,025	383,724
Royalty rate							3%	4%	4%	5%	5%	6%	6%	6%	6%
Royalty (BRAIN License)							1,057	6,545	14,668	23,753	33,286	43,773	54,429	59,878	64,273
Total operating expenses	**169,823**	**44,272**	**50,788**	**56,225**	**62,650**	**213,935**	**226,967**	**278,872**	**340,075**	**395,577**	**434,510**	**463,047**	**489,309**	**507,557**	**524,806**
Income/loss from operations	**(169,823)**	**(44,272)**	**(50,788)**	**(56,225)**	**(62,650)**	**(213,935)**	**(188,728)**	**(88,885)**	**39,625**	**135,267**	**254,208**	**335,832**	**420,841**	**493,408**	**549,406**
Finance (expense)/income	(3,889)	5,344	3,548	3,548	3,548	15,988	(2,249)	(1,595)	(1,757)	(2,792)	(4,514)	(6,811)	(9,581)	(12,691)	(16,097)
Net pre-tax income	**(173,711)**	**(38,928)**	**(47,241)**	**(52,677)**	**(59,102)**	**(197,947)**	**(190,977)**	**(90,480)**	**37,868**	**132,476**	**249,694**	**329,021**	**411,260**	**480,717**	**533,309**
Income tax provision	(1,988)	(273)	(602)	(370)	(415)	(1,660)	(1,340)	(635)	(757)	(6,624)	(22,472)	(46,063)	(74,027)	(100,951)	(111,995)
Net income (loss)	**(175,699)**	**(39,201)**	**(47,843)**	**(53,047)**	**(59,516)**	**(199,606)**	**(192,317)**	**(91,115)**	**37,111**	**125,852**	**227,221**	**282,958**	**337,233**	**379,766**	**421,314**
Earnings per share (non-GAAP)	**(€ 2.97)**	**(€ 0.60)**	**(€ 0.70)**	**(€ 0.73)**	**(€ 0.81)**	**(€ 2.85)**	**(€ 2.58)**	**(€ 1.21)**	**€ 0.45**	**€ 1.51**	**€ 2.69**	**€ 3.31**	**€ 3.90**	**€ 4.34**	**€ 4.77**
Shares Outstanding (Basic)	59,157	65,424	68,073	73,153	73,403	70,013	74,403	75,403	76,403	77,403	78,403	79,403	80,403	81,403	82,403
Shares Outstanding (Diluted)	63,697	70,793	74,073	79,154	79,404	75,856	80,404	81,404	82,404	83,404	84,404	85,404	86,404	87,404	88,404

Source: Company reports; RBC Capital Markets estimates

Cash Flow Tracker															
Cash/equivalents at beginning of year		€ 291,679	€ 246,958	€ 318,282	€ 423,683		€ 371,032	€ 237,642	€ 194,051	€ 281,363	€ 474,116	€ 747,435	€ 1,095,746	€ 1,497,236	€ 1,937,296
Net Cash Used in Operating Activities		(39,201)	(47,843)	(53,047)	(59,516)		(192,317)	(91,115)	37,111	125,852	227,221	282,958	337,233	379,766	421,314
Raises/Payments			132,300	175,000											
Raises/Payments (in Euros)			107,752	151,581											
Price (Actual or Assumption)			*$29.69*	*$62.00*											
# of Additional Shares			*4,456*	*2,823*											
Amortization/Stock-based comp		(5,520)	(13,133)	6,866	6,866		58,926	47,524	50,201	66,902	46,097	65,353	64,257	60,294	60,316
Net Cash End of Year/Quarter		246,958	318,282	423,683	371,032		237,642	194,051	281,363	474,116	747,435	1,095,746	1,497,236	1,937,296	2,418,926

Source: RBC Capital Markets estimates; PHVS Company reports

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Exhibit 3 - PHVS Pipemile

Milestones		
Product	**Event**	**Timeline**
Deucrictibant IR (HAE)	PDUFA	April 23, 2027
Deucrictibant XR (HAE)	NDA Filing for XR	1H27
	Updated data from CHAPTER-3	4Q26+
Deucrictibant IR/XR (AAE)	Topline data from part 1 of CREAATE ph.III study	1Q27

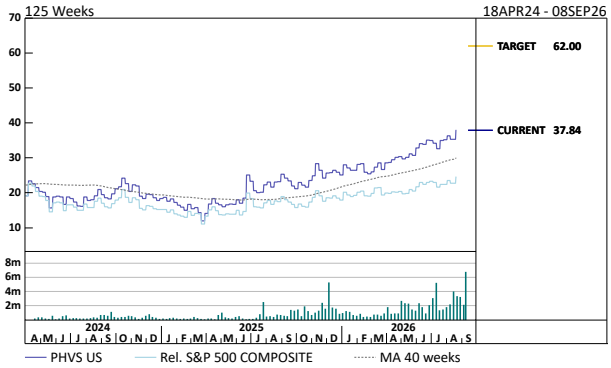
Pipeline		
Product	**Indication**	**Status**
Deucrictibant IR	HAE, AAE - On-demand	Ph.III Complete, Regulatory
Deucrictibant XR	HAE, AAE - Prophylaxis, Pediatrics	Ph.III Complete, Regulatory

Source: RBC Capital Markets estimates; PHVS Company reports

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Target/Upside/Downside Scenarios

Pharvaris N.V.



Source: Bloomberg and RBC Capital Markets estimates for Target

Valuation

Our $62 price target blends DCF (using a 10.5% discount rate and a 1.5% terminal growth rate) and probability-adjusted P/E multiples (30x on 2034E adjusted EPS, discounted at 10.5%) analysis – comparable to other clinical stage biotechs. Our price target supports our Outperform, Speculative Risk rating. We assign a Speculative Risk qualifier given unpredictability of future revenues and expenses, non-revenue-generating status, and stock price volatility that could result in substantial upside/downside swings not anticipated in our valuation.

Upside scenario

Approval of deucrictibant IR for acute HAE attacks and XR formulation for HAE prophylaxis, greater-than-expected long-term commercial uptake, and/or setbacks for competitors would increase our estimated out-year revenue potential for PHVS. In such an upside scenario, we believe fair value, based on DCF and multiples analyses, could be ~$75/share.

Downside scenario

Should deucrictibant IR or XR fail to gain regulatory approval, XR demonstrate safety concerns, or market uptake is slow, our downside scenario suggests that PHVS shares would trade lower, near ~$25/share.

Investment summary

We believe that deucrictibant offers a differentiated profile for both the on-demand and prophylactic setting that could result in meaningful market share over time despite what is increasingly a competitive HAE market, and with other PoC data showing potential in other less competitive areas like AAE-C1INH, we believe there could be a combined ~€1B sales opportunity, which we think could support additional upside.

Key Positives: (1) Deucrictibant IR is de-risked with pivotal trial demonstrating what we believe is a superior profile to another recently launched oral that saw high patient switching from current SoC; (2) CHAPTER-3 trial with XR formulation demonstrated what we think is an overall best-in-class efficacy/safety profile; (3) other prophylactic HAE drugs have sales of >$500M/year, highlighting the opportunity.

Potential Catalysts: (1) PDUFA for IR (April 23, 2027); (2) Topline data from Part 1 of CREAATE AAE trial (1Q27); (3) NDA filing for XR (1H27).

Risks to rating and price target

Key risks: (1) HAE is an increasingly crowded space, limiting market share potential; (2) unexpected safety signals could emerge.

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Key fundamental questions

	Our view
How differentiated is deucrictibant IR's profile as an oral on-demand therapy for HAE?	We believe that KALV's Ekterly's strong adoption since launch so far demonstrates high patient willingness to switch from standard of care icatibant and other injectable/IV options to more convenient oral on-demand options. We believe that across all clinical endpoints of efficacy deucrictibant IR is superior to Ekterly (faster median time to improvement, rapid end of progression, and lower proportion of patients needing rescue medications – all of which KOL feedback suggests is differentiating). Deucrictibant IR has also been well-tolerated in on-demand studies, with further de-risking based on icatibant's extensive use and use of deucrictibant XR in chronic studies, mitigating any other concerns about the less-characterized B2 antagonist mechanism.
What does the CHAPTER-3 readout tell us about deucrictibant XR's competitive positioning in the HAE prophylaxis market?	CHAPTER-3 has, in our view, delivered a best-in-class ph.III result for oral HAE prophylaxis. The 83% mean monthly attack rate reduction vs. placebo in the overall population (87% in HAE Type 1/2) is squarely in injectable-like territory while coming in an oral, once-daily format. We believe the safety profile reinforces differentiation: AEs were balanced with placebo, no GI signal was observed (a known liability of berotralstat), and the single SAE was not treatment-related, though there were some LFT signals that will bear watching. Taken together, deucrictibant XR appears positioned to offer injectable-like efficacy with oral convenience - a combination no currently approved agent delivers - which we believe is the key unlock for meaningful market disruption across both the oral-to-oral switch pool and, potentially, injectable users who have been waiting for a comparable oral option.
What could the commercial dynamic look like for deucrictibant in prophylaxis given the existing therapeutic landscape?	The ongoing CHAPTER-3 ph.III is set to read out later this year (3Q26), and we believe that deucrictibant XR is set up to meaningfully disrupt the prophylaxis setting, based on our KOL feedback and survey data. Although we anticipate significant uptake of XR among new to prophylaxis patients, this opportunity is likely to be incremental given that the market appears to be reaching maturity. However, our survey data suggests that physicians would be willing to switch an average of 35% of patients currently well-managed on Orladeyo to deucrictibant over time, despite what has been described as a "sticky" market, likely due to lower barriers to switching between oral agents. Additionally, given the oral formulation's inherent advantages over injectables, and potential for near-injectable like efficacy, we think that there could be more receptiveness among patients otherwise well controlled on injectables to trial XR, which could further convert the existing injectables market.

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Key sustainability questions

This section is intended to highlight key sustainability discussion points relevant to this company, as well as our views on the outlook. Both the questions we highlight and our responses will evolve over time as the dialogue between management, analysts and investors continues to advance. We welcome any feedback on the topics.

	Our view
What are the most material sustainability issues facing this company?	Like many other biotechs, PHVS faces issues such as green laboratory practices and manufacturing, proper clinical trial conduct, data transparency, diversity of the management team, and hiring and maintaining a diverse set of employees. In the future, we would anticipate that the company will also contend with many of the same sustainability topics that impact the broader sector, chiefly drug pricing, access to medication, and responsible product marketing.
Does the company integrate sustainability considerations into its strategy?	PHVS currently does not have active environmental initiatives, though this is in line with similarly sized biotechs.
What is diversity like at the board/ management level?	As typically seen in biotech companies of similar size, PHVS does not have specific diversity or social initiative programs. Currently 4/9 senior management positions are held by a woman.
Is PHVS developing products to address a significant public health problem?	PHVS is developing and marketing assets to treat rare diseases with substantial disease burden.
How does the company manage data transparency?	The company releases scientific data to a critical and educated audience often in public and peer-reviewed forums and are all made available through their corporate website. Corporate governance documents outlining policies and procedures are made available.

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Company description

Pharvaris (PHVS) is a late-stage biopharmaceutical company focused on the development of novel treatments for bradykinin-mediated angioedema disorders, including hereditary angioedema (HAE) and acquired angioedema (AAE). The company's lead candidate, deucrictibant, is a selective bradykinin B2 receptor antagonist available in both immediate-release (IR) oral capsule formulation for on-demand treatment of acute angioedema attacks and an extended-release (XR) film-coated tablet formulation for QD prophylaxis. PHVS is preparing global marketing authorization applications for deucrictibant IR as an on-demand treatment of HAE attacks, with the pivotal RAPIDe-3 trial demonstrating statistically significant efficacy and tolerability, and also for XR following recent positive CHAPTER-3 data. Additionally, CREAATE, a ph.III study of deucrictibant for both prophylactic and on-demand treatment of AAE-C1INH attacks, is underway. PHVS was founded in 2015 and is headquartered in Zug, Switzerland.

Required disclosures

Conflicts disclosures

The analyst(s) responsible for preparing this research report received compensation that is based upon various factors, including total revenues of the member companies of RBC Capital Markets and its affiliates, a portion of which are or have been generated by investment banking activities of the member companies of RBC Capital Markets and its affiliates.
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Ratings

Outperform (O): Expected to materially outperform sector average over 12 months.
Sector Perform (SP): Returns expected to be in line with sector average over 12 months.
Underperform (U): Returns expected to be materially below sector average over 12 months.
Restricted (R): RBC policy precludes certain types of communications, including an investment recommendation, when RBC is acting as an advisor in certain merger or other strategic transactions and in certain other circumstances.
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The **Speculative** risk rating reflects a security's lower level of financial or operating predictability, illiquid share trading volumes, high balance sheet leverage, or limited operating history that result in a higher expectation of financial and/or stock price volatility.

Distribution of ratings

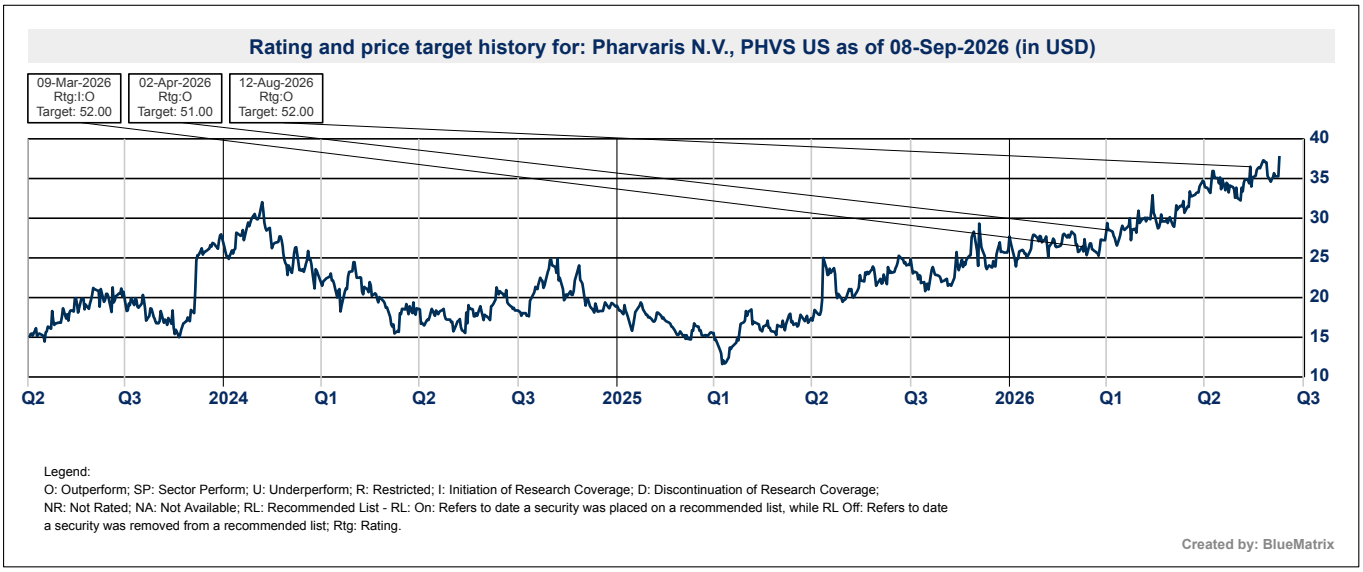
For the purpose of ratings distributions, regulatory rules require member firms to assign ratings to one of three rating categories - Buy, Hold/Neutral, or Sell - regardless of a firm's own rating categories. Although RBC Capital Markets' ratings of Outperform (O), Sector Perform (SP), and Underperform (U) most closely correspond to Buy, Hold/Neutral and Sell, respectively, the meanings are not the same because our ratings are determined on a relative basis.

Distribution of ratings

RBC Capital Markets, Equity Research

As of 30-Jun-2026

			Investment Banking Serv./Past 12 Mos.	
Rating	**Count**	**Percent**	**Count**	**Percent**
BUY [Outperform]	**927**	**57.94**	**290**	**31.28**
HOLD [Sector Perform]	**626**	**39.12**	**161**	**25.72**
SELL [Underperform]	**47**	**2.94**	**3**	**6.38**



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Equity valuation and risks

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Pharvaris N.V.

Valuation

Our $62 price target blends DCF (using a 10.5% discount rate and a 1.5% terminal growth rate) and probability-adjusted P/E multiples (30x on 2034E adjusted EPS, discounted at 10.5%) analysis – comparable to other clinical stage biotechs. Our price target supports our Outperform, Speculative Risk rating. We assign a Speculative Risk qualifier given unpredictability of future revenues

and expenses, non-revenue-generating status, and stock price volatility that could result in substantial upside/downside swings not anticipated in our valuation.

Risks to rating and price target

Key risks: (1) HAE is an increasingly crowded space, limiting market share potential; (2) unexpected safety signals could emerge.

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